INDIVIDUAL CASE REPORT FORM(ICSR)

1. Worldwide Unique Case Identification

Regulatory Authority / Organisation	
Worldwide Unique Case ID	
Receipt Date	
Type of Report	

2. Patient Characteristics

Patient Initials	S. A.
Age at Time of Event	72 years
Gender	Male
Weight	
Allergy Status	No known allergy
Medical History	Hypertension; type 2 diabetes mellitus; old cerebrovascular accident (CVA)
Other Relevant Conditions	Recurrent ischemic strokes (right MCA–PCA watershed infarct and right parietal infarct) with left hemiplegia; old right internal carotid artery distal occlusion; left lower-limb DVT; suspected sepsis with septic shock; chronic kidney disease (CKD); interstitial lung disease/ usual interstitial pneumonia; HFrEF (EF 34%); anemia of chronic disease; history of pulmonary tuberculosis 30 years ago.

3. Adverse Drug Reaction(s)

Primary Suspected Reaction	Hypotension
Date of Onset	19/09/2026
Reported Blood Pressure	90/60 mmHg on 19/09/2026

Outcome / Follow-up	Blood pressure documented as 130/76 mmHg on 20/09/2026 after Bisobis was stopped
Outcome Classification	Improved as documented by the subsequent blood pressure

4. Drug Information

Suspected Drug

Drug Role	Suspected
Medicinal Product Name	Tab. Bisobis 2.5 mg
Active Ingredient	Bisoprolol
Dose / Route / Frequency	2.5 mg once daily; route not explicitly stated in the case
Indication	Heart failure with reduced ejection fraction (HFrEF) and hypertension
Therapy Start Date	18/09/2026
Therapy Stop Date	19/09/2026
Action Taken	Drug discontinued due to hypotension

Concomitant / Previous Medications

Tab. Stiloz 50	Cilostazol; 1-0-1; start date not provided; indication not specified in the case
Tab. Atorva	Atorvastatin; 0-0-1; dose and start date not provided
Tab. Amlodac AT	Amlodipine + atenolol; dose, frequency and start date not provided
Tab. Dytor 10	Torsemide; 1-0-0; start date not provided
Tab. Cospiaq SM	Empagliflozin + sitagliptin + metformin, 25/100/100 mg; half

	tablet morning and evening as recorded; start date not provided

5. Relevant Diagnostic Tests & Laboratory Data

Blood pressure (19/09/2026)	90/60 mmHg
Blood pressure (20/09/2026)	130/76 mmHg

A 72-year-old male patient with hypertension, type 2 diabetes mellitus, old CVA and multiple comorbidities, including recurrent ischemic strokes with left hemiplegia, chronic kidney disease, suspected sepsis with septic shock, interstitial lung disease, left lower-limb DVT, anemia of chronic disease and HFrEF (ejection fraction 34%), was started on Tab. Bisobis (bisoprolol) 2.5 mg on 18/09/2026 for HFrEF and hypertension. His recorded concomitant/previous medicines included Tab. Stiloz, Tab. Atorva, Tab. Amlodac AT, Tab. Dytor and Tab. Cospiaq SM.

On 19/09/2026, the patient developed hypotension, with a documented blood pressure of 90/60 mmHg. Tab. Bisobis 2.5 mg was discontinued in view of the hypotension. Noradrenaline support was documented for hypotension, and blood pressure monitoring every two hours was advised. On 20/09/2026, the blood pressure was documented as 130/76 mmHg.

The temporal relationship between initiation of bisoprolol and the reported hypotension, together with improvement in the recorded blood pressure after withdrawal, supports a possible association. However, the patient had important alternative explanations and contributing factors, including suspected sepsis with septic shock, HFrEF, and concurrent medicines that may affect blood pressure. Therefore, causality should be interpreted cautiously and assessed using the available clinical record.

7. ICH E2A Classification

Seriousness	Potentially serious, given documented hypotension requiring noradrenaline support;
Seriousness Criteria	Noradrenaline support is documented.
Expectedness	Expected

Causality Assessment	Possible

This was done based on case scenario

CASE SCENARIO :

Patient name : S A

Age: 72 year

Gender: Male

Allergy status : No known allergy

Past Medical history : Hypertension, Type 2 Diabetic mellitus, Old CVA

Present illness : Recurrent ais - right mca-pca watershed infarct + right parietal infarct with left hemiplegia, old right ica distal occlusion (only on tab stiloz),dvt left ll, ?sepsis with septic shock, ckd, ild - usual interstitial pneumonia, type 2 dm (hba1c: 7.7%),systemic hypertension, hfref (ef: 34%), Anemia of chronic disease, old pulmonary tb (30 years back)

Past medication history : Tab Stiloz 50 1-0-1
Tab Atorva 0-0-1
Tab Amlodac AT (Amlodipine & Atenolol)
Tab Dytor 10 1-0-0
Tab Cospiaq Sm (Empa+Sita+MF) 25/100/100mg 1/2-0-1/2

A 72 Year old male patient was Started on 18/9/2026 Tab Bisobis (Bisoprolol) 2.5mg For HFrEF and Hypertension. Later on 19/9/2026 he developed Hypotension. The drug was stopped. On 20/09/2026 Blood pressure came to normal 130/76mmhg.

Laboratory data :

Physical examination: on 19/9/2026 Blood pressure: 90/60 mmhg ,

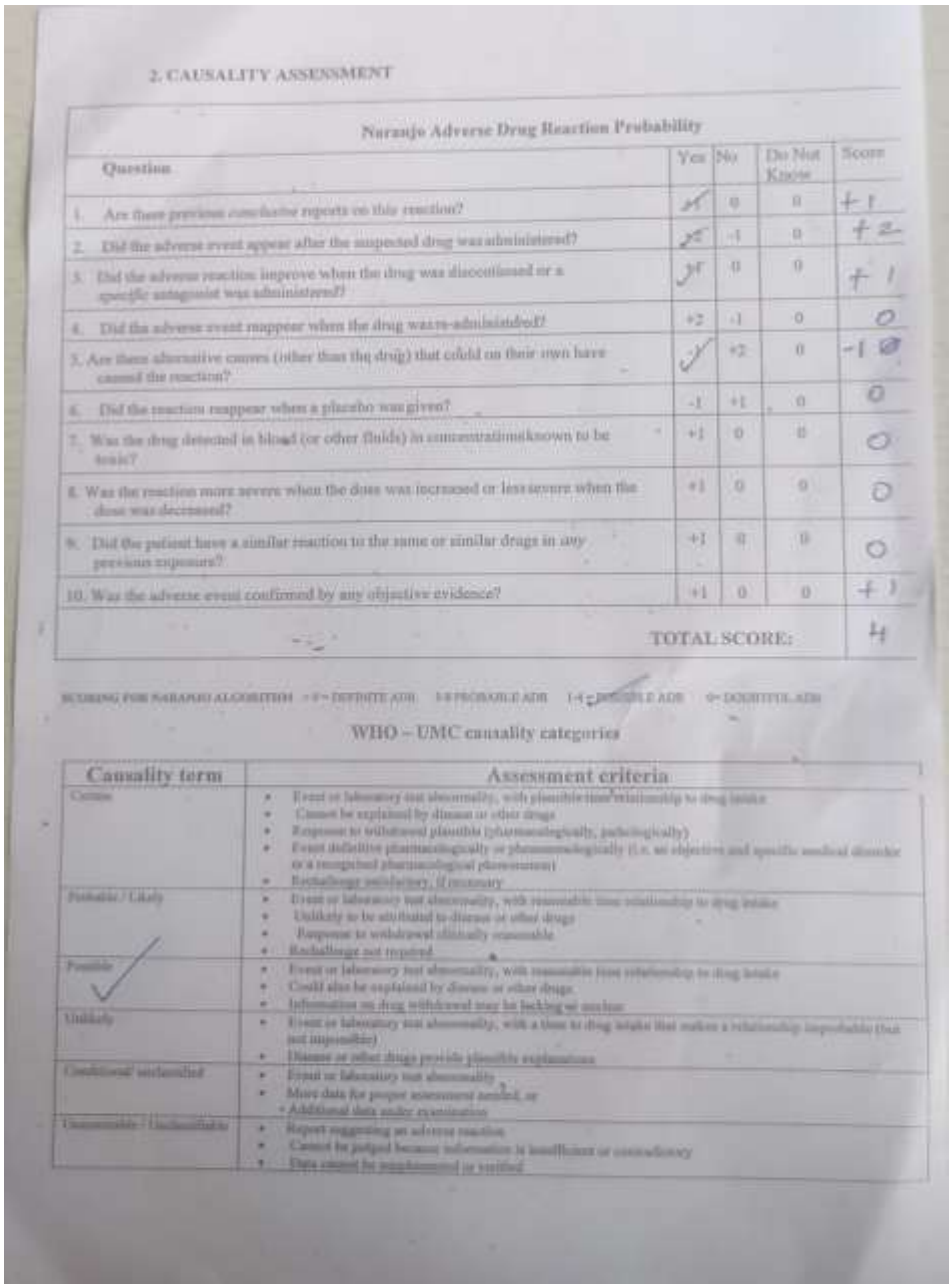
20/9/2026 Blood presuure:130/76mmhg

Management:

Tab Bisobis 2.5 mg was discontinued due to hypotension. Noradrenaline support was documented for hypotension. Blood pressure monitoring was advised every 2 hours.

On 20/09/2026 Blood pressure came to normal 130/76mmhg.

Causality assessment done using Naranjo scale



2. CAUSALITY ASSESSMENT

Naranjo Adverse Drug Reaction Probability

Question	Yes	No	Do Not Know	Score
1. Are there previous conclusive reports on this reaction?	✓	0	0	+1
2. Did the adverse event appear after the suspected drug was administered?	✓	-1	0	+2
3. Did the adverse reaction improve when the drug was discontinued or a specific antagonist was administered?	✓	0	0	+1
4. Did the adverse event reappear when the drug was re-administered?	+2	-1	0	0
5. Are there alternative causes (other than the drug) that could on their own have caused the reaction?	✓	+2	0	-1
6. Did the reaction reappear when a placebo was given?	-1	+1	0	0
7. Was the drug detected in blood (or other fluids) in concentrationsknown to be toxic?	+1	0	0	0
8. Was the reaction more severe when the dose was increased or less severe when the dose was decreased?	+1	0	0	0
9. Did the patient have a similar reaction to the same or similar drugs in any previous exposure?	+1	0	0	0
10. Was the adverse event confirmed by any objective evidence?	+1	0	0	+1
			TOTAL SCORE:	4

SCORING FOR NARANJO ALGORITHM: ≥9= DEFINITE ADR; 5-8 PROBABLE ADR; 1-4 = POSSIBLE ADR; 0= DOUBTFUL ADR

WHO – UMC causality categories

Causality term	Assessment criteria
Certain	• Event or laboratory test abnormality, with plausible time relationship to drug intake • Cannot be explained by disease or other drugs • Response to withdrawal plausible (pharmacologically, pathologically) • Event definitive pharmacologically or phenomenologically (i.e. an objective and specific medical disorder or a recognized pharmacological phenomenon) • Rechallenge satisfactory, if necessary
Probable / Likely	• Event or laboratory test abnormality, with reasonable time relationship to drug intake • Unlikely to be attributed to disease or other drugs • Response to withdrawal clinically reasonable • Rechallenge not required
Possible ✓	• Event or laboratory test abnormality, with reasonable time relationship to drug intake • Could also be explained by disease or other drugs • Information on drug withdrawal may be lacking or unclear
Unlikely	• Event or laboratory test abnormality, with a time to drug intake that makes a relationship improbable (but not impossible) • Disease or other drugs provide plausible explanations
Conditional/ unclassified	• Event or laboratory test abnormality • More data for proper assessment needed, or • Additional data under examination
Unassessable / Unclassifiable	• Report suggesting an adverse reaction • Cannot be judged because information is insufficient or contradictory • Data cannot be supplemented or verified